

Proposed Prescribing Information
For the use of a Registered Medical Practitioner or a Hospital or a Laboratory

TORQUITE SR 300 MG TABLETS
QUETIAPINE FUMARATE SUSTAINED RELEASE TABLETS 300MG

BRAND OR PRODUCT NAME

TORQUITE SR 300 MG TABLETS

NAME AND STRENGTH OF ACTIVE SUBSTANCE(S)

TORQUITE SR 300 MG TABLETS

Each film coated sustained release tablet contains: Quetiapine
Fumarate equivalent to Quetiapine300mg

Colors: Titanium Dioxide and Yellow oxide of Iron

PRODUCT DESCRIPTION

TORQUITE SR 300 MG TABLETS

Pale yellow coloured, capsule shaped biconvex film coated tablets, plain on both sides.

DOSAGE FORM

Film coated sustained release tablet

PHARMACODYNAMICS

Pharmacotherapeutic group: Antipsychotics

ATC code: N05A H04

Mechanism of action:

Quetiapine is an atypical antipsychotic agent. Quetiapine and the active human plasma metabolite, norquetiapine interact with a broad range of neurotransmitter receptors. Quetiapine and norquetiapine exhibit affinity for brain serotonin (5HT₂) and dopamine D₁- and D₂-receptors. It is this combination of receptor antagonism with a higher selectivity for 5HT₂ relative to D₂-receptors, which is believed to contribute to the clinical antipsychotic properties and low extrapyramidal side effect (EPS) liability of quetiapine compared to typical antipsychotics. Additionally, norquetiapine has high affinity at serotonin 5HT₁ receptors. Quetiapine and norquetiapine also have high affinity at histaminergic and adrenergic α_1 -receptors, with a lower affinity at adrenergic α_2 -receptors. Quetiapine has no appreciable affinity at muscarinic or benzodiazepine receptors. Quetiapine is active in tests for antipsychotic activity, such as conditioned avoidance. It also blocks the action of dopamine agonists, measured either behaviourally or electrophysiologically, and elevates dopamine metabolite concentrations, a neurochemical index of D₂-receptor blockade. The extent to which the norquetiapine metabolite contributes to the pharmacological activity of quetiapine in humans is not

known.

Pharmacokinetic properties

Quetiapine is well absorbed following oral administration. Quetiapine is approximately 83% bound to plasma proteins. Steady-state peak molar concentrations of the active metabolite norquetiapine are 35% of that observed for quetiapine.

The pharmacokinetics of quetiapine and norquetiapine are linear across the approved dosing range. The kinetics of quetiapine does not differ between men and women.

Quetiapine ER achieves peak plasma concentrations at approximately 6 hours after administration (T_{max}). Quetiapine ER displays dose-proportional pharmacokinetics for doses of up to 800 mg administered once daily. The maximum plasma concentration (C_{max}) and the area under the plasma concentration-time curve (AUC) for Quetiapine ER administered once daily are comparable to those achieved for the same total daily dose of immediate-release quetiapine fumarate IR administered twice daily. The elimination half lives of quetiapine and norquetiapine are approximately 7 and 12 hours, respectively.

The mean clearance of quetiapine in the elderly is approximately 30 to 50% lower than that seen in adults aged 18 to 65 years.

There are no clinically relevant differences in the observed apparent oral clearance (CL/F) and exposure of quetiapine between subjects with schizophrenia and bipolar disorder.

The mean plasma clearance of quetiapine was reduced by approximately 25% in subjects with severe renal impairment (creatinine clearance less than 30 ml/min/1.73m²), but the individual clearance values are within the range for normal subjects. The average molar dose fraction of free quetiapine and the active human plasma metabolite norquetiapine is <5% excreted in the urine.

Quetiapine is extensively metabolised by the liver, with parent compound accounting for less than 5% of unchanged drug-related material in the urine or faeces, following the administration of radiolabelled quetiapine. Approximately 73% of the radioactivity is excreted in the urine and 21% in the faeces. The mean quetiapine plasma clearance decreases by approx. 25% in persons with known hepatic impairment (stable alcohol cirrhosis). As quetiapine is extensively metabolised by the liver, elevated plasma levels are expected in the population with hepatic impairment. Dose adjustments may be necessary in these patients.

CYP3A4 is the primary enzyme responsible for cytochrome P450 mediated metabolism of quetiapine. Norquetiapine is primarily formed and eliminated via CYP3A4.

Quetiapine and several of its metabolites (including norquetiapine) were found to be weak inhibitors of human cytochrome P450 1A2, 2C9, 2C19, 2D6 and 3A4 activities in vitro. In vitro CYP inhibition is observed only at concentrations approximately 5 to 50 fold higher than those observed at a dose range of 300 to 800 mg/day in humans. Based on these in vitro results, it is unlikely that co-administration of quetiapine with other drugs will result in clinically significant drug inhibition of cytochrome P450 mediated metabolism of the other drug. From animal studies it appears that quetiapine can induce cytochrome P450 enzymes. In a specific interaction study in psychotic patients, however, no increase in the cytochrome P450 activity was found after administration of quetiapine.

Quetiapine SR should be taken at least one hour before a meal.

INDICATION

Torquite SR is indicated for the:

- Treatment of schizophrenia.
- Preventing relapse in stable schizophrenic patients who have been maintained on Torquite

SR.

- Treatment of moderate to severe manic episodes in the framework of bipolar disorder.
- Treatment of major depressive episodes in bipolar disorder.
- Major depressive disorder Treatment of recurrent major depressive disorder (MDD) in patients who are intolerant of, or who have an inadequate response to alternative therapies.
- Prevention of recurrence in patients with bipolar disorder, in patients whose manic or depressive episode has responded to quetiapine treatment.
- Maintenance treatment of bipolar I disorder as adjunctive therapy to lithium or divalproex.

RECOMMENDED DOSAGE

Quetiapine SR should be administered once daily, without food. The tablets should be swallowed whole and not split, chewed or crushed.

Adults:

For the treatment of schizophrenia and moderate to severe manic episodes associated with bipolar disorder

Quetiapine SR should be administered at least one hour before a meal. The daily dose at the start of therapy is 300 mg on Day 1 and 600 mg on Day 2.

The recommended daily dose is 600 mg, however if clinically justified the dose may be increased to 800 mg daily. The dose should be adjusted within the effective dose range of 400 mg to 800 mg per day, depending on the clinical response and tolerability of the patient. For maintenance therapy in schizophrenia no dosage adjustment is necessary.

For the treatment of depressive episodes associated with bipolar disorder

Quetiapine SR should be administered at bedtime. Quetiapine should be administered once daily at bedtime. The total daily dose for the first four days of therapy is 50 mg (Day 1), 100 mg (Day 2), 200 mg (Day 3) and 300 mg (Day 4). The recommended daily dose is 300 mg. No additional benefit was reported in the 600 mg group compared to the 300 mg group. Individual patients may benefit from a 600 mg dose. Doses greater than 300 mg should be initiated by physicians experienced in treating bipolar disorder. In individual patients, in the event of tolerance concern, dose reduction to a minimum of 200 mg could be considered.

Maintenance treatment of bipolar I disorder as adjunctive therapy to lithium or divalproex

While there is no evidence available to specifically address how long the patient treated with Quetiapine SR should remain on it, maintenance of efficacy in Bipolar I Disorder was demonstrated with Quetiapine IR tablets (administered twice daily totalling 400 to 800 mg per day) as adjunct therapy to lithium or divalproex. Generally, in the maintenance phase, patients continued on the same dose on which they were stabilized during the stabilization phase

For preventing recurrence in bipolar disorder

For preventing recurrence of manic, mixed or depressive episode in bipolar disorder, patients who have responded to Quetiapine SR for acute treatment of bipolar disorder should continue on Quetiapine SR at the same dose administered at bedtime. Quetiapine SR dose can be adjusted depending on clinical response and tolerability of the individual patient within the dose range

of 300mg to 800 mg/day. It is important that the lowest effective dose is used for maintenance therapy.

Recurrent major depressive disorder

When treating recurrent MDD in patients who are intolerant of, or who have an inadequate response to alternative therapies, treatment should be initiated either by the treating psychiatrist or by the general practitioner after consultation with the psychiatrist. TORQUITE SR should be administered once daily in the evening. Initial dosing should begin at 50 mg on Day 1 and 2, increased to 150 mg on Day 3 and 4. The usual effective dose in MDD is 150 mg. Further adjustments can be made upwards or downwards within the recommended dose range of 50 mg to 300 mg depending upon the clinical response and tolerability of the patient.

Patients who have not responded to TORQUITE SR after 6 weeks treatment for MDD should have treatment re-evaluated.

For maintenance therapy in MDD in patients who have responded to acute treatment, the effective dose during initial treatment should be continued. It is generally recommended that responding patients be continued beyond the acute response, but at the lowest possible dose needed to maintain remission. The dose can be adjusted within the recommended dose range depending upon the clinical response and tolerability of the patient. Patients should be periodically reassessed to determine the need for maintenance treatment.

Long-term safety of TORQUITE SR in MDD has not been systematically evaluated (>52 weeks). Thus, the physician who elects to use TORQUITE SR in the treatment of MDD should use TORQUITE SR for the shortest time that is clinically indicated. When lengthier treatment is indicated, the physician must periodically re-evaluate the long-term usefulness of the drug for the individual patient keeping in mind the long-term risks.

Switching from Quetiapine immediate-release tablets:

For more convenient dosing, patients who are currently being treated with divided doses of immediate release Quetiapine tablets may be switched to Quetiapine SR at the equivalent total daily dose taken once daily. Individual dosage adjustments may be necessary.

Elderly:

As with other antipsychotic, Quetiapine SR should be used with caution in the elderly, especially during the initial dosing period. The rate of dose titration of Quetiapine SR may need to be slower, and the daily therapeutic dose lower, than that used in younger patients. The mean plasma clearance of quetiapine was reduced by 30 % to 50 % in elderly patients when compared to younger patients.

Elderly patients should be started on 50 mg/day. The dose can be increased in increments of 50 mg/day to an effective dose, depending on the clinical response and tolerability of the individual patient.

In elderly patients with MDD, initial dosing should begin at 50 mg on Days 1-3, the dose can be increased to 100 mg on Day 4, 150 mg on Day 8 and then up to 300 mg depending on clinical response and tolerability.

Mode of administration

Oral

CONTRAINDICATIONS

- Hypersensitivity to the active substance or to any of the excipients of this product.
- Concomitant administration of cytochrome P450 3A4 inhibitors, such as HIV-

protease inhibitors, azole-antifungal agents, erythromycin, clarithromycin and nefazodone, is contraindicated.

WARNINGS AND PRECAUTIONS

As quetiapine is indicated for the treatment of schizophrenia, bipolar disorder and add-on treatment of major depressive episodes in patients with MDD, the safety profile should be considered with respect to the individual patient's diagnosis and the dose being administered.

Long-term efficacy and safety in patients with MDD has not been reported as add-on therapy, however long-term efficacy and safety has been reported in adult patients as monotherapy.

Hyperglycemia and Diabetes Mellitus

Hyperglycemia in some cases extreme and associated with ketoacidosis or hyperosmolar coma or death, has been reported in patients treated with atypical antipsychotics. Assessment of the relationship between atypical antipsychotics use and glucose abnormalities is complicated by the possibility of an increased background risk of diabetes mellitus in patients with schizophrenia and the increasing incidence of diabetes mellitus in the general population. Given this confounders, the relationship between atypical antipsychotic use and hyperglycemia-related adverse events is not completely understood. However, epidemiological studies suggest an increased risk of treatment emergent hyperglycemia-related events in patients treated with the atypical antipsychotics. Precise risk estimates for hyperglycemia-related adverse events in patients treated with atypical antipsychotics are not available.

Patients with an established diagnosis of diabetes mellitus who are started on atypical antipsychotics should be monitored regularly for worsening of glucose control. Patients with risk factors for diabetes mellitus (e.g. obesity, family history of diabetes) who are starting treatment with atypical antipsychotics should undergo fasting blood glucose testing at the beginning of treatment and periodically during treatment. Any patient treated with atypical antipsychotics should be monitored for symptoms of hyperglycemia including polydipsia, polyuria, polyphagia, and weakness. Patients who develop symptoms of hyperglycemia during treatment with atypical antipsychotics should undergo fasting blood glucose testing. In some cases, hyperglycemia has resolved when the atypical antipsychotic was discontinued; however, some patients required continuation of anti-diabetic treatment despite discontinuation of the suspect drug.

Children and adolescents (10 to 17 years of age)

Quetiapine SR is not recommended for use in children and adolescents below 18 years of age, due to a lack of data to support use in this age group. Certain adverse events occurred at a higher frequency in children and adolescents compared to adults (increased appetite, elevations in serum prolactin and extrapyramidal symptoms) and one was reported that has not been previously reported in adult studies (increases in blood pressure). Changes in thyroid function tests have also been reported in children and adolescents.

Furthermore, the long-term safety implications of treatment with quetiapine on growth and maturation have not been reported beyond 26 weeks. Long-term implications for cognitive and behavioural development are not known.

It was reported that there was an increased incidence of extrapyramidal symptoms (EPS) in quetiapine treatment compared to placebo for schizophrenia and bipolar mania in children and adolescent patients

Suicide/suicidal thoughts or clinical worsening:

Depression is associated with an increased risk of suicidal thoughts, self-harm and suicide (suicide related events). This risk persists until significant remission occurs. As improvement may not occur during the first few weeks or more of treatment, patients should be closely monitored until such improvement occurs. It is general clinical experience that the risk of suicide may increase in the early stages of recovery. In addition, physicians should consider the potential risk of suicide-related events after abrupt cessation of quetiapine treatment, due to the known risk factors for the disease being treated.

Other psychiatric conditions for which quetiapine is prescribed can also be associated with an increased risk of suicide related events. In addition, these conditions may be co-morbid with major depressive episodes. The same precautions observed when treating patients with major depressive episodes should therefore be observed when treating patients with other psychiatric disorders.

Patients with a history of suicide related events, or those exhibiting a significant degree of suicidal ideation prior to commencement of treatment are known to be at greater risk of suicidal thoughts or suicide attempts, and should receive careful monitoring during treatment.

Close supervision of patients and in particular those at high risk should accompany drug therapy especially in early treatment and following dose changes. Patients (and caregivers of patients) should be alerted about the need to monitor for any clinical worsening, suicidal behaviour or thoughts and unusual changes in behaviour and to seek medical advice immediately if these symptoms present.

Extrapyramidal symptoms:

It was reported that adult patients treated with quetiapine was associated with an increased incidence of extrapyramidal symptoms (EPS) compared to placebo in patients treated for major depressive episodes in bipolar disorder and major depressive disorder.

The use of quetiapine has been associated with the development of akathisia, characterised by a subjectively unpleasant or distressing restlessness and need to move often accompanied by an inability to sit or stand still. This is most likely to occur within the first few weeks of treatment. In patients who develop these symptoms, increasing the dose may be detrimental.

Tardive Dyskinesia:

Tardive dyskinesia is a syndrome of potentially irreversible, involuntary, dyskinetic movements that may develop in patients treated with antipsychotic drugs including quetiapine. If signs and symptoms of tardive dyskinesia appear, dose reduction or discontinuation of quetiapine should be considered. The symptoms of tardive dyskinesia can worsen or even arise after discontinuation of treatment.

Somnolence and dizziness:

Quetiapine treatment has been associated with somnolence and related symptoms, such as sedation. Bipolar depression patients and patients with major depressive episodes in MDD experiencing somnolence of severe intensity may require more frequent contact for a minimum of 2 weeks from onset of somnolence, or until symptoms improve and treatment discontinuation may need to be considered.

Quetiapine treatment has been associated with orthostatic hypotension and related dizziness

which, like somnolence has onset usually during the initial dose-titration period. This could increase the occurrence of accidental injury (fall), especially in the elderly population. Therefore, patients should be advised to exercise caution until they are familiar with the potential effects of the medication.

Cardiovascular:

Quetiapine should be used with caution in patients with known cardiovascular disease, cerebrovascular disease, or other conditions predisposing to hypotension. Quetiapine may induce orthostatic hypotension especially during the initial dose-titration period and therefore dose reduction or more gradual titration should be considered if this occurs. A slower titration regimen could be considered in patients with underlying cardiovascular disease.

Seizures:

As with other antipsychotics, caution is recommended when treating patients with a history of seizures.

Neuroleptic Malignant Syndrome:

Neuroleptic malignant syndrome has been associated with antipsychotic treatment, including quetiapine. Clinical manifestations include hyperthermia, altered mental status, muscular rigidity, autonomic instability, and increased creatinine phosphokinase. In such an event, quetiapine should be discontinued and appropriate medical treatment given.

Severe neutropenia:

Severe neutropenia (neutrophil count $<0.5 \times 10^9/L$) has been uncommonly reported. There is no apparent dose relationship. During post-marketing experience, resolution of leucopenia and/or neutropenia has followed cessation of therapy with quetiapine. Possible risk factors for neutropenia include pre-existing low white cell count (WBC) and history of drug induced neutropenia. Quetiapine should be discontinued in patients with a neutrophil count $<1.0 \times 10^9/L$. Patients should be observed for signs and symptoms of infection and neutrophil counts followed (until they exceed $1.5 \times 10^9/L$).

Interactions:

Concomitant use of quetiapine with a strong hepatic enzyme inducer such as carbamazepine or phenytoin substantially decreases quetiapine plasma concentrations, which could affect the efficacy of quetiapine therapy. In patients receiving a hepatic enzyme inducer, initiation of quetiapine treatment should only occur if the physician considers that the benefits of quetiapine outweigh the risks of removing the hepatic enzyme inducer. It is important that any change in the inducer is gradual, and if required, replaced with a non-inducer (e.g. sodium valproate).

Weight:

Weight gain has been reported in patients who have been treated with quetiapine, and should be monitored and managed as clinically appropriate as in accordance with utilised antipsychotic guidelines.

Lipids:

Increases in triglycerides, LDL and total cholesterol, and decreases in HDL cholesterol have been reported with quetiapine. Lipid changes should be managed as clinically appropriate.

Metabolic Risk:

Given the observed changes in weight, blood glucose (see hyperglycaemia) and lipids seen in

clinical studies, there may be possible worsening of the metabolic risk profile in individual patients, which should be managed as clinically appropriate.

Pancreatitis:

Pancreatitis has been reported in clinical trials and during the post marketing experience, however a causal relationship has not been established. Among the post marketing reports, many patients had factors which are known to be associated with pancreatitis such as increased triglycerides (see ‘Special warnings and precautions for use – Lipids’), gallstones, and alcohol consumption.

QT Prolongation:

QT prolongation was reported with quetiapine at the therapeutic doses and in overdose. As with other antipsychotics, caution should be exercised when quetiapine is prescribed in patients with cardiovascular disease or family history of QT prolongation. Also caution should be exercised when quetiapine is prescribed either with medicines known to increase QT interval, or with concomitant neuroleptics, especially in the elderly, in patients with congenital long QT syndrome, congestive heart failure, heart hypertrophy, hypokalaemia or hypomagnesaemia.

Withdrawal:

Acute withdrawal symptoms such as nausea, vomiting, insomnia, headache, diarrhoea, dizziness and irritability have been described after abrupt cessation of high doses of quetiapine. Gradual withdrawal over a period of at least one to two weeks is advisable.

Elderly patients with dementia-related psychosis:

Quetiapine is not approved for the treatment of dementia-related psychosis. An approximately 3-fold increased risk of cerebrovascular adverse events has been reported in the dementia population with some atypical antipsychotics. The mechanism for this increased risk is not known. An increased risk cannot be excluded for other antipsychotics or other patient populations. Quetiapine should be used with caution in patients with risk factors for stroke.

Hepatic effects:

If jaundice develops, quetiapine should be discontinued.

Concomitant Illness:**Dysphagia:**

Dysphagia (see ‘Undesirable effects’) and aspiration have been reported with quetiapine. Although a causal relationship with aspiration pneumonia has not been established, quetiapine should be used with caution in patients at risk for aspiration pneumonia.

Constipation and intestinal obstruction:

Constipation represents a risk factor for intestinal obstruction. Constipation and intestinal obstruction have been reported with quetiapine (see ‘Undesirable effects’). This includes fatal reports in patients who are at higher risk of intestinal obstruction, including those that are receiving multiple concomitant medications that decrease intestinal motility and/or may not report symptoms of constipation.

Venous thromboembolism:

Cases of venous thromboembolism (VTE) have been reported with antipsychotic drugs. Since patients treated with antipsychotics often present with acquired risk factors for VTE, all possible risk factors for VTE should be identified before and during treatment with quetiapine and preventive measures undertaken.

Lactose:

TORQUITE SR tablets contain lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency, or glucose-galactose malabsorption should not take this medicine.

INTERACTIONS WITH OTHER MEDICAMENTS

Given the primary central nervous system effects of quetiapine, quetiapine should be used with caution in combination with other centrally acting drugs and alcohol.

Cytochrome P450 CYP3A4 is the enzyme that is primarily responsible for the cytochrome P450 mediated metabolism of quetiapine. In an interaction study in healthy volunteers, concomitant administration of quetiapine (dosage of 25 mg) with ketoconazole, a CYP3A4 inhibitor, caused a 5- to 8-fold increase in the AUC of quetiapine. On the basis of this, concomitant use of quetiapine with CYP3A4 inhibitors is contraindicated. It is also not recommended to consume grapefruit juice while on quetiapine therapy.

Co-administration of carbamazepine significantly increased the clearance of quetiapine. This increase in clearance reduced systemic quetiapine exposure (as measured by AUC) to an average of 13% of the exposure during administration of quetiapine alone; although a greater effect was seen in some patients. As a consequence of this interaction, lower plasma concentrations can occur, which could affect the efficacy of quetiapine therapy. Co-administration of quetiapine and phenytoin (another microsomal enzyme inducer) caused a greatly increased clearance of quetiapine by approx. 450%. In patients receiving a hepatic enzyme inducer, initiation of quetiapine treatment should only occur if the physician considers that the benefits of quetiapine outweigh the risks of removing the hepatic enzyme inducer. It is important that any change in the inducer is gradual, and if required, replaced with a non-inducer (e.g. sodium valproate).

The pharmacokinetics of quetiapine were not significantly altered by co-administration of the antidepressants imipramine (a known CYP2D6 inhibitor) or fluoxetine (a known CYP3A4 and CYP2D6 inhibitor).

The pharmacokinetics of quetiapine were not significantly altered by co-administration of the antipsychotics risperidone or haloperidol. Concomitant use of quetiapine and thioridazine caused an increased clearance of quetiapine of approx. 70%.

The pharmacokinetics of quetiapine were not altered following co-administration with cimetidine.

The pharmacokinetics of lithium were not altered when co-administered with quetiapine. The pharmacokinetics of sodium valproate and quetiapine were not altered to a clinically relevant extent when co-administered.

Formal interaction studies with commonly used cardiovascular drugs have not been performed.

Caution should be exercised when quetiapine is used concomitantly with drugs known to cause electrolyte imbalance or to increase QTc interval.

There have been reports of false positive results in enzyme immunoassays for methadone and tricyclic antidepressants in patients who have taken quetiapine. Confirmation of questionable immunoassay screening results by an appropriate chromatographic technique is recommended.

PREGNANCY AND LACTATION

Pregnancy and lactation

To be used with caution in pregnancy & breast feeding.

Neonates exposed to antipsychotic drugs during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hypertonia, hypotonia, tremor, somnolence, respiratory distress and feeding disorder in these neonates. These complications have varied in severity; while in some cases symptoms have been self-limited, in other cases neonates have required intensive care unit support and prolonged hospitalisation.

Quetiapine tablet should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus.

The safety and efficacy of quetiapine during human pregnancy have not yet been established. Up to now there are no indications for harmfulness in animal tests; possible effects on the foetal eye have not been examined, though. Therefore, quetiapine should only be used during pregnancy if the benefits justify the potential risks. Following pregnancies in which quetiapine was used, neonatal withdrawal symptoms were observed.

The degree to which quetiapine is excreted into human milk is unknown. Women who are breast-feeding should therefore be advised to avoid breast-feeding while taking quetiapine.

ADVERSE EFFECTS / UNDESIRABLE EFFECTS

The most commonly reported Adverse Drug Reactions (ADRs) with quetiapine are somnolence, dizziness, dry mouth, mild asthenia, constipation, tachycardia, orthostatic hypotension and dyspepsia.

As with other antipsychotics, weight gain, syncope, neuroleptic malignant syndrome, leucopenia, neutropenia and peripheral oedema, have been associated with quetiapine. The incidences of ADRs associated with quetiapine therapy, are tabulated below.

Blood and lymphatic system disorders
Leucopenia Eosinophilia, Thrombocytopenia Neutropenia
Immune system disorders
Hypersensitivity Anaphylactic
Endocrine disorders
Hyperprolactinaemia Inappropriate antidiuretic hormone secretion
Metabolism and nutritional disorders
Increased appetite Hyponatraemia Diabetes Mellitus
Psychiatric disorders
Abnormal dreams and nightmares Suicidal ideation and suicidal behaviour Somnambulism and other related events
Nervous system disorders

Dizziness, somnolence, headache Syncope, Extrapyramidal symptoms, Dysarthria Seizure, restless leg syndrome, Tardive dyskinesia
Cardiac disorders
Tachycardia, Palpitations
Eye disorders
Vision blurred
Vascular disorders
Orthostatic hypotension Venous thromboembolism
Respiratory, thoracic and mediastinal disorder
Rhinitis, dyspnea, Sleep apnoea* *Atypical antipsychotic drugs, such as Quetiapine, have been associated with cases of sleep apnoea, with or without concomitant weight gain. In patients who have a history of or are at risk for sleep apnoea, Torquise should be prescribed with caution.
Gastrointestinal disorders
Dry mouth Constipation, dyspepsia, vomiting Dysphagia
Hepato-biliary disorders
Jaundice, Hepatitis
Skin and subcutaneous tissue disorders
Angioedema, Stevens-Johnson syndrome
Musculoskeletal and connective tissue disorders
Rhabdomyolysis
Reproductive system and breast disorders
Sexual dysfunction Priapism, Galactorrhoea, breast swelling, menstrual disorder
Renal and Urinary Disorders:
Urinary retention
General disorders and administration site conditions
Withdrawal (discontinuation) symptoms Mild asthenia, peripheral oedema, irritability, pyrexia Neuroleptic malignant syndrome, hypothermia
Investigations
Elevations in serum triglyceride levels. Elevations in total cholesterol (predominantly LDL cholesterol), Decreases in HDL cholesterol, weight gain, decreased haemoglobin. Elevations in serum transaminases (ALT, AST), decreased neutrophil count, blood glucose increased to hyperglycaemic levels, decreases in total T4, decreases in free T4, decreases in total T3, increases in TSH. Elevations in gamma-GT levels, Platelet count decreased, QT prolongation, decreases in free T3. Elevations in blood creatine phosphokinase

Children and adolescents (10 to 17 years of age)

The same ADRs described above for adults should be considered for children and adolescents. The following table summarizes ADRs that occur in a higher frequency category in children and adolescent patients (10-17 years of age) than in the adult population or ADRs that have not been identified in the adult population.

Metabolism and nutritional disorders
Increased appetite
Investigations
Elevations in prolactin, increases in blood pressure
Nervous system disorders
Extrapyramidal symptoms
General disorders and administration site conditions
Irritability

OVERDOSE AND TREATMENT

Fatal outcome has been reported following an acute overdose at 13.6 grams, and in post-marketing on doses as low as 6 grams of quetiapine alone. However, survival has also been reported following acute overdoses of up to 30 grams. In post marketing experience, there have been very rare reports of overdose of quetiapine alone resulting in death or coma or QT-prolongation.

Patients with pre-existing severe cardiovascular disease may be at an increased risk of the effects of overdose.

In general, reported signs and symptoms were those resulting from an exaggeration of the drug's known pharmacological effects, i.e., drowsiness and sedation, tachycardia and hypotension.

Management of overdose

There is no specific antidote to quetiapine. In cases of severe signs, the possibility of multiple drug involvement should be considered, and intensive care procedures are recommended, including establishing and maintaining a patent airway, ensuring adequate oxygenation and ventilation, and monitoring and support of the cardiovascular system. Whilst the prevention of absorption in overdose has not been investigated, gastric lavage can be indicated in severe poisonings and if possible to perform within one hour of ingestion. The administration of activated charcoal should be considered. In cases of quetiapine overdose refractory hypotension should be treated with appropriate measures such as intravenous fluids and/or sympathomimetic agents (epinephrine and dopamine should be avoided, since beta stimulation may worsen hypotension in the setting of quetiapine-induced alpha blockade). Close medical supervision and monitoring should be continued until the patient recovers

STORAGE CONDITIONS

Store below 30°C.

PACKAGING AVAILABLE

Quetiapine Fumarate Sustained Release Tablets are packed in Alu-Alu blister (OPA/ALU/PVC- ALU) of 10 tablets. Such blisters containing 10 tablets are packed into boxes of 10's and 60's. Not all presentations or all pack sizes may be marketed.

DATE OF REVISION OF PACKAGE INSERT

22-January-2019

NAME AND ADDRESS OF MANUFACTURER



TORRENT PHARMACEUTICALS LTD.

Indrad 382 721, District:

Mehsana INDIA.